February 23, 2026

Thomas Keane, MD, MBA
Assistant Secretary for Technology Policy
Office of the National Coordinator for Health Information Technology
Department of Health and Human Services
Washington, DC 20201

RE: Request for Information: HHS Health Sector AI RFI (0955-AA13)

Sent electronically via: http://www.regulations.gov

Dear Assistant Secretary Keane,

Corewell Health appreciates the opportunity to provide comments on the "Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care." Corewell Health is a Michigan-based nonprofit integrated health system with a team of 65,000+ dedicated people including more than 12,000 physicians and advanced practice providers and more than 16,000 nurses providing care and services in 21 hospitals and 300+ outpatient locations. In addition, as an integrated health system, Corewell Health includes Priority Health, a health plan that insures more than 1.3 million lives. Corewell Health is not only Michigan's largest health system but also Michigan's largest private employer. Through experience and collaboration, we are reimagining a better, more equitable model of health and wellness. We submit these comments through the lens of an integrated health system.

We appreciate that HHS is considering a multifaceted approach to encouraging the responsible adoption of artificial intelligence (AI) through regulation, reimbursement, and research and development. **In our experience, the most significant barriers to AI adoption are not regulatory restrictions primarily but uncertainty.** Health systems require clear evidence of safety and effectiveness, defined accountability structures between vendors and purchasers, sustainable payment pathways, and modernized data standards to support deployment. Absent these elements, adoption slows not due to resistance, but due to prudent risk management. While AI has the potential to be a positive tool in transforming health care, we appreciate that HHS is being deliberative in developing a framework that establishes trust among all stakeholders—patients, providers, payers, policymakers, and developers.

Supporting activities to promote innovative and effective AI use in clinical care

One of the most critical barriers preventing widespread adoption of AI in health care may not be regulatory burdens but the need for assurance of the utility, efficiency, and reliability of existing AI products for both clinical and non-clinical uses. To build off our submission to the Request for Public Comment on the use of AI-enabled medical devices, AI for clinical purposes should demonstrate exceptional performance in safety, effectiveness, and reliability to ensure it enhances patient care without introducing new risks.

We would encourage a consensus process that meets the standards set by both providers and payers before patients can access them for safe and effective care. From a provider perspective, testing should validate clinical performance, support evidence-based decision-making, and safeguard patient outcomes. Metrics of interest include diagnostic concordance with human experts, time to result, alert precision and recall, and rates of false positives or negatives. From a health plan perspective, real-world evidence is essential to confirm medical necessity, justify reimbursement, and prevent unnecessary costs. Any proposed metric should be weighted by clinical consequences, with higher weighting for patient safety and outcome improvement compared to workflow efficiency. The safety, effectiveness, and reliability of AI-enabled medical devices must be measured through clinical performance validation, user accuracy, workflow impact, and safety reporting.

In addition, evaluating real-world clinical use performance should be based on review of high-quality studies published in peer-reviewed journals, not anecdotal observations or marketing assertions. The data must be longitudinal, incorporating short-term performance post-deployment, and ongoing monitoring as practice patterns evolve. The data should demonstrate effect intervals extending well beyond the typical disease progression timeframe to demonstrate durability of effects. For example, if a patient's cancer typically doubles in six months, we expect evidence of durable benefit, such as remission lasting two to four years.

Such evaluations would not only be beneficial to large integrated health systems like ours but also crucial to smaller providers to be confident in adopting AI solutions. Without clear guidelines, the federal government risks exacerbating a new type of digital divide within the health care sector with safety-net providers and their patients being unable to harness AI's transformative power.

Finally, HHS should review with stakeholder input whether existing regulatory frameworks require modernization to reflect AI-enabled care delivery. Areas for review could include HIPAA Privacy and Security Rule interpretations related to AI training and secondary data use, alignment of requirements for AI-enabled clinical decision support tools, and clarification of Stark and anti-kickback implications for subscription-based AI models integrated into clinical workflows. Greater clarity in these areas would support innovation while maintaining patient protection.

Creating a framework for responsibility between AI vendors and purchasers

Relatedly, HHS should develop a clear, comprehensive framework that identifies an AI vendor's responsibilities to ensure legal and regulatory compliance to give providers of all sizes some assurances for its AI products.

As a non-profit entity, we have dedicated resources to our Artificial Intelligence Center of Excellence (AI COE) at a considerable cost. This level of investment may not be feasible for smaller organizations, especially safety-net providers in underserved or rural areas, or sustainable in the long term even for large health systems. For externally developed AI solutions, a framework should require how vendors (i.e., any vendor integrating AI into their products) will demonstrate regulatory compliance to their customers. It would be helpful for key health and non-health agencies to provide clear guidance on how to effectively demonstrate and document sufficient compliance as AI technology continues to evolve.

In addition to ensuring compliance, the performance of these tools is exceedingly important to validate and monitor – especially when the patient population and data some of these tools were trained on does not accurately reflect the patient population and data the tools will be used to treat. The responsibility for this type of validation and monitoring needs to be a shared responsibility between an AI vendor and the purchasing organization to ensure safe and effective use across all patient populations.

Similar to the prior considerations for supporting AI evaluation, we also believe a framework for responsibility here would be beneficial across the health sector to be confident in adopting AI solutions. Without such clarity, the federal government risks exacerbating a new type of digital divide within the health care sector with safety-net providers and their patients being unable to harness AI's transformative power. Worse yet, there becomes an increasing risk of predatory AI vendors taking advantage of smaller, less resourced organizations in ways that both harm their financial standing and their patients and leads to skepticism among patients and providers on AI utilization.

For non-medical device AI tools, novel governance issues arise related to liability allocation, indemnification, model transparency, bias mitigation, and post-deployment monitoring. HHS should clarify expectations regarding shared accountability between AI vendors and health care organizations, particularly where models influence clinical decision-making but are not regulated as medical devices. Clear guidance regarding documentation standards, auditability, and safe harbor protections for organizations deploying AI under structured governance frameworks would meaningfully reduce uncertainty.

Considering payment policy for AI utilization

Similar to our comments on the Calendar Year Medicare Physician Fee schedule, we would encourage HHS to consider appropriate Medicare payment policies for responsible AI utilization. For example, some digital technologies use a subscription model: we use subscription-model algorithms to help with detection in imaging such as CT and X-rays for mammography or bone fractures or in scheduling follow-up services from imaging and clinical decision support for clinicians on various workflows that can connect such workflows with orders, order sets, and clinical best practice advisories. Such services enable better care management, especially for chronic conditions, by providing clinicians with greater access to up-to-date, evidence-based clinical standards and helping integrate resulting orders into electronic health records. As a subscription, the vendor can update the service to keep it continuously informed with new clinical and scientific research on best practices and foster continuous quality improvement.

HHS should be aware that providers still need to allocate internal staff resources to use the software. Payment policy should recognize that high-value AI deployment may shift work rather than eliminate it, requiring investments in training, governance, cybersecurity, and monitoring. Reimbursement pathways should align incentives toward measurable outcome improvement and total cost of care reduction rather than volume-based utilization of AI-enabled services. This may mean incorporating not only clinical costs but also time need for aforementioned items such as training, cybersecurity, and maintenance.

Supporting providers to develop a governance structure

HHS should include public-private partnerships to help providers develop the AI governance infrastructure to manage risk and provide guardrails for deployment. Such a structure can help provide transparency to patients and consumers on how AI is used and can be beneficial in healthcare. For example, the Corewell Health AI COE oversees the process for onboarding and monitoring AI technologies going forward. Our goal is to develop a responsible and sustainable approach for using AI technology and similar tools in ways that improve our work while also protecting our patient and plan member information when there is high value and low risk.

Other hospitals and providers may need resources and guidance to adopt a similar approach to our AI COE, and we too have appreciated tools developed by the federal government to ensure we are adopting best practices. Such tools should be flexible enough to be adopted by different types of stakeholders. For example, as required under the National Artificial Intelligence Initiative Act of 2020 (P.L. 116-283), the AI Risk Management Framework developed by the National Institute of Standards and Technology (NIST) has been a valuable resource. The NIST framework helps ensure that we are managing the potential risks of AI and promoting trustworthiness among staff, patients, and our community. We appreciate that it is a voluntary tool that is not specific either to sector or use-case.

Having such a transparent governance structure to show how AI is used in care delivery is essential to building public trust and sustaining it. Patients should have clarity regarding when AI informs clinical decisions, assurances regarding bias monitoring and privacy protections, and appropriate avenues for recourse when concerns arise. Responsible adoption requires not only technological safeguards but clear communication.

Developing Interoperability and Data Standards

Enhanced interoperability is foundational to scaling AI in clinical care. Continued advancement of FHIR-based APIs; expansion of USCDI to include structured imaging metadata, genomic information, and standardized social risk data; and development of benchmarking datasets for model validation would significantly widen market opportunities and accelerate research. AI tools are only as effective as the quality and standardization of the data on which they operate.

Developing a talent pool

We encourage HHS and other federal agencies to consider how to develop a pipeline for more AI and cybersecurity professionals, including incentives to work in the health care sector. In adopting potential AI solutions, health plans and health systems are developing more in-depth reviews and implementation processes, which require substantial resources and administrative capacity. Health care providers operate on narrow margins yet must compete globally for talent with entities that have much larger access to capital.

Applying a regulatory framework to direct-to-consumer health AI tools

Finally, patients and consumers are seeking out AI tools for their health needs on their own initiative, and developers are launching new products to help them organize and understand their health data. Such tools have the potential to empower patients to make healthier choices and manage their own health conditions but also lead to potential harm such as confusion, misinformation, and inappropriate use of patient information. For example, HHS and the Federal Trade Commission may need to modify existing resources to help developers and consumers understand their responsibilities under existing privacy and security laws. Many consumers do not understand that depending on who they share their personal health information with, HIPAA protections may not necessarily apply.

We appreciate that HHS is considering some of these issues through Medicare demonstrations such as the Advancing Chronic Care with Effective, Scalable Solutions (ACCESS) Model. But many issues, including reimbursement, privacy, and security, may be outside of HHS's current regulatory authority and require congressional action. As HHS considers whether and how to incorporate direct-to-consumer AI tools into the broader health care system, it should reactions from stakeholders—providers, plans, and most importantly patients—on a proposed regulatory framework.

Conclusion

Thank you for the opportunity to share our perspective. Should you have any questions regarding these comments or if you would like any additional information, please contact Oliver Kim, Senior Director of Public Policy, Corewell Health Government Relations & Public Policy, oliver.kim@corewellhealth.org.

Sincerely,

Signed by:

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